THE ROLE OF INFORMATION IN REDUCING DRUG USE AMONG FISHERMEN IN HOMABAY COUNTY

BY

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DECLARATION

This research project is my original work and has not been submitted for a degree or any other award in any other university.

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DEDICATION

This research project is dedicated to my beloved parents Jackson Obwo and Wilfride Obwo for their unwavering support, encouragement and sacrifices throughout my education. Your love and belief in me have brought me this far.

I also dedicate this work to the fishing communities of Homabay County, whose struggles and resilience inspired this study. May this work contribute to improving your lives and well-being.

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ABSTRACT

Drug and substance abuse among fishing communities in Kenya has become a major public health concern with far-reaching consequences for individuals, families, and the economy. This study investigated the role of information in reducing drug use among fishermen in Homabay County, Kenya. The study was guided by two objectives: to examine the factors contributing to high drug use among fishermen, and to explore the role of information in reducing substance use. The study employed a mixed-methods research design combining quantitative and qualitative approaches. A sample of 120 fishermen was drawn from three beaches (Mbita, Sori, and Ringiti) using convenience sampling. Data were collected using structured questionnaires, three Focus Group Discussions (FGDs), and three Key Informant Interviews with Beach Management Unit (BMU) leaders. Quantitative data were analyzed using descriptive statistics (frequencies and percentages), while qualitative data were analyzed thematically. The findings revealed that drug use is extremely high among fishermen, with 72.5% reporting current use. Alcohol was the most commonly used substance (93.3%), followed by marijuana (63.8%). The main reasons for drug use were coping with stress (72.4%), peer pressure (64.8%), and needing energy for work (49.5%). Regarding information, the study found that fishermen primarily rely on peers (74.2%) and radio (55.8%) for information, but trust BMU leaders (34.2%) and health workers (29.2%) most. Awareness of NACADA was low (35.8%), and only 31.7% knew about the Alcoholic Drinks Act. Only 19.2% received drug prevention information regularly. Among those who received information, 48.4% reported reducing or stopping drug use. However, barriers such as peer pressure (65.0%), addiction (51.7%), and stress (48.3%) prevented action. Fishermen preferred receiving information through beach meetings with BMU leaders (63.3%) and radio programs in Dholuo (56.7%). The study concludes that information plays a significant but limited role in reducing drug use and must be accompanied by support services addressing addiction, stress, and environmental factors. Recommendations include strengthening information delivery through trusted sources, providing accessible rehabilitation services, and addressing the easy availability of drugs at beaches.

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ABBREVIATIONS AND ACRONYMS

AIDS - Acquired Immunodeficiency Syndrome

BMU - Beach Management Unit

BCC - Behavioral Change Communication

CHV - Community Health Volunteer

FGD - Focus Group Discussion

HBM - Health Belief Model

HIV - Human Immunodeficiency Virus

KII - Key Informant Interview

NACADA - National Authority for the Campaign Against Alcohol and Drug Abuse

NACOSTI - National Commission for Science, Technology and Innovation

NGO - Non-Governmental Organization

NPS - Novel Psychoactive Substances

SCT - Social Cognitive Theory

SPSS - Statistical Package for Social Sciences

UNODC - United Nations Office on Drugs and Crime

OPERAIONAL DEFINITIONS OF TERMS

Addiction – A condition in which a person is unable to stop using a substance despite wanting to, often experiencing withdrawal symptoms when they try to stop.

Alcohol – A beverage containing ethanol, including traditional brews (busaa, chang'aa) and commercial beers that is consumed for recreational purposes and is the most commonly abused substance in this study.

Barrier – Any factor that prevents fishermen from acting on information about drugs or from successfully reducing or stopping drug use.

Beach Management Unit (BMU) – A community-based organization comprising fishing stakeholders responsible for managing beach activities, resolving conflicts, and representing the fishing community.

Credibility – The degree to which fishermen perceive an information source as trustworthy, reliable, and worth believing.

Drug – Any substance other than food that when taken into the body alters its function physically or psychologically. In this study, this includes alcohol, marijuana, tobacco, and prescription drugs used non-medically.

Fisherman – A person who engages in fishing activities as their main source of livelihood, including boat owners, crew members, and fishmongers operating at the beach.

Focus Group Discussion (FGD) – A qualitative research method involving a group of 8-10 fishermen discussing specific topics guided by a facilitator.

Harmful alcohol use – A pattern of alcohol consumption that causes damage to physical or mental health, as defined by the World Health Organization.

Information – Facts, knowledge, or messages provided or received concerning drugs, their effects, and ways to prevent or reduce substance use.

Information source – The person, organization, or medium through which fishermen receive information about drugs (e.g., peers, radio, health workers, BMU leaders).
Key Informant – A person with specialized knowledge about the fishing community and drug issues, such as BMU leaders, who provides in-depth information through interviews.

Marijuana (bhang) – A psychoactive drug derived from the cannabis plant, commonly smoked and used recreationally by fishermen.

NACADA – The National Authority for the Campaign Against Alcohol and Drug Abuse, a Kenyan government agency responsible for coordinating national efforts to prevent and control substance abuse.

Peer pressure – Influence from members of one's social group to engage in certain behaviors, in this context, pressure to use drugs to fit in or be accepted.

Perceived barrier – An individual's assessment of the obstacles to changing their drug use behavior, as defined by the Health Belief Model.

Perceived severity – An individual's belief about the seriousness of the consequences of drug use, as defined by the Health Belief Model.

Perceived susceptibility – An individual's belief about their personal risk of experiencing harm from drug use, as defined by the Health Belief Model.

Prevalence – The proportion of fishermen in the study population who use drugs, expressed as a percentage.

Rehabilitation – The process of helping a person recover from addiction through medical treatment, counseling, and support services.

Self-efficacy – An individual's confidence in their ability to successfully reduce or stop drug use, as defined by both the Health Belief Model and Social Cognitive Theory.

Stress – A state of mental or emotional strain resulting from adverse or demanding circumstances, identified as a primary reason for drug use among fishermen.

Substance abuse – The harmful or hazardous use of psychoactive substances, including alcohol and illicit drugs.

Thematic analysis – A qualitative data analysis method that involves identifying, analyzing, and reporting patterns (themes) within the data.

Triangulation – The use of multiple data sources or methods (quantitative and qualitative) to enhance the validity of research findings.

CHAPTER ONE

INTRODUCTION

This chapter presents the background of the study, statement of the problem, objectives, research questions, significance of the study, justification of the study, scope of the study, and limitations of the study. The study focuses on the role of information access in reducing drug use among fishermen in Homa Bay County.

1.1 Background of the Study

Drug and substance abuse remains a significant global public health challenge affecting millions of people worldwide. According to Our World in Data (2026), a considerable proportion of the global population suffers from alcohol and drug use disorders, contributing to major health, social, and economic problems. The global epidemiology of drug use shows increasing cases of injecting drug use and associated diseases such as HIV, tuberculosis, and viral hepatitis in many parts of the world (Open Source Pharma Foundation, 2023). The emergence of new psychoactive substances has further complicated the global drug problem, with studies indicating increasing prevalence across different regions of the world (Springer, 2023). These trends demonstrate the importance of effective prevention strategies, including access to information and awareness programs to educate communities about the dangers of drug use.

In Kenya, drug and substance abuse has become a growing public health and social concern affecting individuals across different sectors of society. Reports indicated that over 1.3 million Kenyans required rehabilitation services due to drug and substance abuse (NACADA, 2026; KBC Digital, 2026), highlighting the magnitude of the problem in the country. The increasing number of drug abuse victims prompted the government to introduce interventions such as specialized narcotics courts and strengthened anti-drug campaigns (The Standard, 2026a). Despite these efforts, the prevalence of drug abuse continued to rise, with many individuals requiring treatment and rehabilitation services (The Standard, 2026b). Access to reliable information and awareness programs has therefore been a critical strategy for preventing drug use and promoting healthier behaviors within communities

At the local level, drug and substance abuse has been reported in fishing communities around Homa Bay County. Fishing communities are vulnerable to risky behaviors due to the nature of fishing activities, including long working hours, unstable income, and social influences. Studies conducted among fishermen in Homa Bay and Siaya Counties showed that behavioral change communication programs significantly influenced high-risk behaviors such as drug and alcohol use (Akobi, Oyore & Otieno, 2022; Akobi, Oyore & Otieno, 2023). Similarly, research by Awuonda (2014a) found that substance abuse among fisherfolk in Homa Bay County was influenced by peer pressure, economic challenges, and the availability of drugs within fishing communities. These studies emphasized the role of community education and information dissemination in reducing substance abuse (Awuonda, 2014b). Access to accurate information and awareness programs therefore played a critical role in shaping attitudes and behaviors toward drug use among fishermen.

1.2 Statement of the Problem

Substance abuse remained a major public health and socio-economic challenge globally and nationally. Worldwide, drug and substance use disorders contributed significantly to preventable illnesses and deaths, particularly among vulnerable populations (Our World in Data, 2023). The increasing use of drugs and emerging psychoactive substances continued to pose serious health and social problems across many countries (Open Source Pharma Foundation, 2022).

In Kenya, drug and substance abuse became a growing concern affecting individuals across different communities. It was estimated that about 1.3 million Kenyans required treatment and rehabilitation services due to alcohol and drug use disorders (KBC Digital, 2026). However, rehabilitation facilities remained limited, leaving many individuals without access to proper treatment and support. Fishing communities in Homa Bay County were particularly vulnerable to substance abuse due to demanding working conditions, peer influence, and easy access to drugs. Studies showed that a large proportion of fisherfolk in the area abused substances such as alcohol and marijuana despite low income levels (Awuonda, 2014a; Awuonda, 2014b). Substance abuse among fishermen was also associated with other risky behaviors that negatively affected their health and livelihoods (Akobi et al., 2022).

Despite the existence of laws and interventions aimed at controlling drug abuse, many fishermen had limited awareness of these measures and the dangers associated with substance use (Awuonda, 2014b). Although previous studies examined behavioral change communication among fishermen, limited research focused specifically on the role of information in reducing drug use within fishing communities. This study examined the role of information in reducing drug use among fishermen in Homa Bay County.

1.3 Purpose of the Study

The study examined the role of information in reducing drug use among fishermen in Homa Bay County. It determined how access to information, awareness of the dangers of drug use, and exposure to information sources influenced substance use behavior among fishermen. It also explored the sources of information available to fishermen and assessed whether increased awareness contributed to behavioral change and reduction of drug use within fishing communities.

1.4 Objectives of the Study

1.4.1 General Objective

To assess the role of information access in reducing drug use among fishermen in Homa Bay County.

1.4.2 Specific Objectives

- To examine the factors contributing to high drug use among fishermen.
- To establish existing information gaps related to drug-use prevention.

1.5 Research Questions

- What factors contributed to high drug use among fishermen?
- What information gaps existed in drug-use prevention?

1.6 Significance of the Study

The study was significant to policymakers, community organizations, healthcare providers, fishermen, and researchers. The findings provided useful information to government agencies such as the National Authority for the Campaign Against Alcohol and Drug Abuse and the Ministry of Health in designing effective awareness and prevention programs aimed at reducing drug use among fishermen.

Organizations and community groups working in Homa Bay County used the findings to improve behavioral change communication strategies and develop targeted interventions within fishing communities. The study also benefited fishermen and Beach Management Units by promoting awareness of the dangers of drug use and encouraging community-based initiatives that supported healthier lifestyles. In addition, healthcare providers improved counseling and outreach services for individuals affected by substance abuse (KBC Digital, 2026). Finally, the study contributed to the existing academic literature on substance abuse and information use in vulnerable communities and provided a basis for further research in similar settings (Springer, 2026).

1.7 Scope of the Study

The study was conducted in Homa Bay County, Kenya, focusing on selected fishing communities along the shores of Lake Victoria, particularly in Mbita Constituency, due to its high fishing activity and documented substance abuse among fisherfolk (Awuonda, 2014a; Akobi et al., 2022). The research targeted fishermen, including boat owners, crew members, and fishmongers, as well as key informants such as Beach Management Unit (BMU) leaders, community health volunteers, and local health workers to provide broader community perspectives. The study examined the role of information in reducing drug use, including: types and patterns of substances used, sources and credibility of information, awareness of health, social, economic, and legal consequences, and barriers to using information to change behavior (Akobi et al., 2022; Awuonda, 2014b). Clinical assessment, medical diagnosis, and rehabilitation outcomes were not included, as the focus was on information access and its influence on behavior. A mixed-methods approach was employed, combining surveys with focus group discussions (FGDs) and key informant interviews (KIIs) to capture both quantitative patterns and qualitative insights. Data collection took place over two weeks, providing a cross-sectional snapshot of drug use and information access among fishermen in Homa Bay County.

1.8 Justification of the Study

This study was justified by the high prevalence of substance abuse among fishermen in Homa Bay County, which negatively affects their health, income, and social wellbeing. Limited access to accurate information and awareness programs has contributed to continued high-risk behaviors despite existing interventions (Awuonda, 2014b). Understanding how information influences drug use can guide policymakers, NGOs, and healthcare providers in designing targeted interventions and awareness campaigns that promote behavioral change and improve community health outcomes.

CHAPTER TWO

LITERATURE REVIEW

2.1 Introduction

This chapter reviews related literature in line with the objectives of the study. It examines factors contributing to high drug use among fishermen and the role of information in reducing substance use, with the global, national, and local perspectives integrated under each objective. The review highlights relevant findings from existing studies and identifies gaps that justified the current research.

2.2 Factors Contributing to High Drug Use among Fishermen

Drug and substance use among fishing communities is influenced by several interconnected factors operating at global, national, and local levels. Globally, substance use disorders affect a large proportion of the population and are a major cause of preventable illness and death (Our World in Data, 2023; World Health Organization, 2024). Research shows that marginalized and high-risk populations often experience higher levels of substance abuse. The emergence of new psychoactive substances has further complicated the global drug problem, increasing health risks and challenges for public health systems (Springer, 2023).

At the national level in Kenya, drug and substance abuse remains a major public health concern. Reports indicate that over 1.3 million Kenyans require rehabilitation services due to alcohol and drug use disorders (KBC Digital, 2026). Although the government has introduced measures such as anti-drug campaigns and specialized narcotics courts (The Standard, 2026a; The Standard, 2026b), substance abuse persists. Studies suggest that limited access to reliable information, low awareness of laws such as the Alcoholic Drinks Act, and limited knowledge of regulatory bodies like NACADA contribute to continued drug use (Awuonda, 2014b). Much of the research in Kenya has focused on youth and urban populations, leaving occupational groups such as fishermen less studied.

Locally in Homa Bay County, fishing communities face specific vulnerabilities that contribute to substance abuse. A study conducted in Mbita Constituency found that drugs are easily accessible within fishing communities, with 68.8% of respondents reporting easy availability. Alcohol was

the most commonly abused substance (62.3%), followed by marijuana (28.1%) (Awuonda, 2014a). The study also showed that many fishermen spend a significant portion of their income on drugs, despite having generally low earnings (Awuonda, 2014b). Several factors explain this high level of substance use. Occupational stress is a major factor, as fishermen often work long and irregular hours, engage in night fishing, and operate in risky lake environments. These conditions lead to fatigue, stress, and anxiety, which some fishermen attempt to manage through substance use (Awuonda, 2014a).
Economic instability also contributes to the problem because fishing income is unpredictable and seasonal, encouraging some individuals to use substances as a coping mechanism (Awuonda, 2014b). Peer influence and social networks within fishing communities further normalize drug use. Younger fishermen often adopt drinking habits from older colleagues, and alcohol consumption is commonly used as a social activity that strengthens group bonds (Akobi et al., 2022). In addition, bars and alcohol vendors are often located near landing sites, increasing exposure and access to substances. Information gaps also play a significant role. Studies indicate that many fishermen lack awareness of relevant laws and institutions addressing substance abuse. For example, a large proportion of fisherfolk were unaware of the Alcoholic Drinks Act and the role of NACADA (Awuonda, 2014b). Limited education and lack of alternative recreational activities in beach communities also contribute to substance use. Research has further linked substance abuse among fishermen to other high-risk behaviors. Alcohol consumption has been associated with risky sexual practices, including multiple sexual partners and inconsistent condom use, which increases the risk of HIV transmission (Akobi et al., 2022).

In summary, high drug use among fishermen is influenced by multiple factors including occupational stress, economic instability, peer influence, easy access to substances, and limited access to reliable information. Addressing these factors requires comprehensive interventions that combine information dissemination, community awareness, and supportive policies.

2.3 The Role of Information in Reducing Drug Use

Information and awareness programs are widely recognized as important tools in preventing substance abuse and promoting harm reduction. Information can influence people's attitudes, beliefs, and behaviors toward drug use when it is properly designed and effectively communicated.

Globally, information plays a central role in drug control strategies and public health interventions. The emergence of new psychoactive substances and the changing global drug market has increased the need for accurate and timely information to educate communities about the risks of drug abuse (Springer, 2023). Studies show that information campaigns can increase awareness of drug-related harms and influence risk perception. However, research also suggests

that information is most effective when combined with broader behavioral change strategies that address social norms and environmental factors (Open Source Pharma Foundation, 2022).
At the national level in Kenya, institutions such as NACADA have implemented public awareness campaigns, school outreach programs, and media initiatives aimed at educating the public about the dangers of drug and substance abuse (KBC Digital, 2026). Despite these efforts, many individuals in high-risk communities remain unaware of existing laws and regulatory bodies addressing substance abuse (Awuonda, 2014b). The government has also introduced specialized narcotics courts to strengthen legal enforcement and public awareness (The Standard, 2026a).
Studies in Kenya have demonstrated that behavioral change communication programs can influence risky behaviors. For example, research on HIV prevention has shown that targeted information interventions can improve condom use and reduce risky sexual behaviors (Akobi, Oyore & Otieno, 2022; Akobi, Oyore & Otieno, 2023). Although these studies focus mainly on health behaviors, they indicate that well-structured information can influence behavior even in high-risk environments.

Locally in Homa Bay County, research on the role of information in reducing drug use among fishermen remains limited. However, behavioral change communication programs conducted in Homa Bay and Siaya Counties have shown that access to accurate information can reduce high-risk behaviors associated with substance use, particularly risky sexual practices linked to alcohol consumption (Akobi et al., 2022; Akobi et al., 2023). These programs combined information delivery with counseling and peer education, indicating that information is more effective when delivered through interactive and credible channels. Despite these efforts, fishing communities continue to face significant information gaps. Fishermen often have limited access to structured information sources such as libraries, health education programs, and community information centers. Low literacy levels, limited digital skills, and geographical distance from information services further restrict access to reliable information (Akobi et al., 2023). Studies have also revealed that many fishermen are unaware of the Alcoholic Drinks Act and the role of NACADA in addressing substance abuse (Awuonda, 2014b). In many fishing communities, information about drugs is mainly obtained from informal sources such as peers, bar owners, and social networks. While peer networks can help spread information, they can also promote misinformation and normalize substance abuse (Akobi et al., 2023). As a result, many fishermen remain unaware of available rehabilitation services and the long-term social, health, and economic consequences of drug use. Information can influence drug use behavior in several ways. It increases awareness of the health, economic, and legal consequences of substance abuse, helps change attitudes toward drug use, provides guidance on how to avoid or reduce substance use, and corrects misinformation circulating within communities (Awuonda, 2014b; Akobi et al., 2022). When fishermen receive accurate and relevant information, they can make more informed decisions and even influence others within their social networks. However, important gaps remain in existing research. Many studies examine information as part of broader interventions,

making it difficult to determine its direct impact on drug use behavior. In addition, limited research has explored which types of information and which communication channels are most effective in reaching fishermen. Barriers such as strong social norms, economic pressures, and addiction also limit the effectiveness of information-based interventions. This study therefore examines the role of information in reducing drug use among fishermen in Homa Bay County. Specifically, it explores how access to information influences substance use behavior, identifies the main sources of information available to fishermen, and examines barriers that prevent information from leading to behavioral change. The findings are expected to contribute to the development of more effective and targeted information programs for fishing communities.

2.5 Theoretical Framework

This study is guided by two complementary theories that provide a lens for understanding the factors contributing to drug use among fishermen and the role of information in reducing substance abuse. These theories are the Health Belief Model (HBM) and the Social Cognitive Theory (SCT) . Together, they offer a comprehensive framework for examining how individual beliefs, social influences, and environmental factors interact to shape health behaviors related to drug use.

2.5.1 The Health Belief Model (HBM)

The Health Belief Model, originally developed in the 1950s by social psychologists Hochbaum, Rosenstock and Kegels, is one of the most widely used theoretical frameworks for understanding health behavior. The model was initially designed to explain why people failed to participate in preventive health programs, but it has since been applied to a wide range of health behaviors including substance use, HIV prevention and treatment adherence.
This study applies the Health Belief Model to examine how fishermen's beliefs about drug use influence their behaviors and how information can modify these beliefs.

2.5.2 Social Cognitive Theory (SCT)

Social Cognitive Theory (SCT)**,** developed by Albert Bandura, explains that human behavior is shaped by the interaction between personal factors, environmental influences, and behavior itself, a concept known as reciprocal determinism (Bandura, 1986). The theory emphasizes that people learn not only through direct experience but also through observing others and interpreting the consequences of their actions. Several key concepts of SCT are relevant to this study. Observational learning (modeling) suggests that individuals adopt behaviors by observing others, especially those they admire or interact with regularly. In fishing communities, fishermen may imitate peers who engage in drug or alcohol use, particularly when such behaviors appear

socially accepted (Breuer et al., 2019). Outcome expectations also influence behavior, as individuals are more likely to engage in substance use if they believe it will produce positive outcomes such as relaxation, stress relief, or social bonding (Bandura, 2004). Another important concept is self-efficacy, which refers to an individual's confidence in their ability to control or change their behavior. Higher self-efficacy has been associated with a greater likelihood of reducing harmful substance use (Naigino et al., 2023). Additionally, environmental factors such as peer influence, alcohol availability, and advertising in fishing communities can reinforce substance use behaviors (Brathwaite et al., 2024). In this study, SCT helps explain how drug and substance use among fishermen may be shaped by social interactions, environmental conditions, and personal beliefs. By understanding these factors, information interventions can be designed to promote positive role models, reshape outcome expectations about drug use, and strengthen fishermen's confidence in their ability to reduce or stop substance use.

2.5.3 Conceptual Framework

Based on the theoretical frameworks discussed above, the conceptual framework illustrates the relationship between information-related factors and drug use behaviors among fishermen. The independent variables in this study include information-related factors such as access to information sources (health workers, media, peers, and community leaders), the content of information on health risks, legal consequences, and economic impacts of drug use, the credibility of information sources, frequency of information exposure, and awareness of drug laws and regulatory bodies such as NACADA and the Alcoholic Drinks Act.

The intervening variables consist of individual and social factors that mediate the relationship between information and behavior. These include individual beliefs such as perceived susceptibility, perceived severity, perceived benefits, and perceived barriers as explained in the Health Belief Model (HBM); self-efficacy, which refers to confidence in one's ability to change behavior as emphasized in both the Health Belief Model and Social Cognitive Theory (SCT); outcome expectations regarding the consequences of drug use; social norms and peer influence; and environmental factors such as the availability of drugs and alcohol advertising.

The dependent variable is drug use behavior among fishermen, including the types of substances used (such as alcohol and marijuana), the frequency and quantity of use, expenditure on substances, and efforts to reduce or stop drug use.

The framework therefore suggests that information influences drug use behavior both directly and indirectly through its effect on individual beliefs, self-efficacy, and outcome expectations, which shape behavioral decisions. However, this relationship may be moderated by social and environmental factors. For example, even when fishermen receive accurate information about drug risks, continued peer influence and the availability of alcohol around landing sites may make behavior change more difficult. The study therefore examined these relationships to determine how information can be effectively designed and disseminated to reduce drug and substance use among fishermen in Homa Bay County

Diagrammatic Representation of the Conceptual Framework

INFORMATION VARIABLES

(Independent)

Access to sources
Content of information
Source credibility
Frequency of exposure
Legal awareness

INTERVENING VARIABLES
(Mediating factors)

Health Belief Model:
Perceived susceptibility
Perceived severity
Perceived benefits
Perceived barriers
Social Cognitive Theory:
Self-efficacy
Outcome expectations
Social norms
Environmental factor

DRUG USE BEHAVIORS
(Dependent)

Types of substances
Frequency of use
Quantity consumed
Expenditure on drugs
Reduction efforts

Figure 2.1 Conceptual Framework

2.6 Research Gap

Despite the growing body of literature on substance use globally and in Kenya, several important gaps existed that informed this study. Many previous studies examined substance abuse as part of broad interventions that combined information with counseling, peer support, or structural changes, making it difficult to isolate the independent effect of information on drug use behaviors (Naigino et al., 2023; Akobi et al., 2022). In addition, limited evidence existed on which types of information such as health risks, legal consequences or economic impacts and which information sources were most credible and effective for fishermen. Fishermen also remained an under-researched population in Kenya, as most substance abuse studies focused on youth, students and urban populations, while fishing communities in the Lake Victoria region have unique social and occupational characteristics that influence substance use patterns (Breuer et al., 2019). Global literature further indicated limited research from African contexts, highlighting gaps in geographically representative evidence (Springer, 2022). Methodologically, many studies relied on cross-sectional designs that described patterns of substance use but did not establish causal relationships between information exposure and behavior change (Awuonda, 2014). Furthermore, limited qualitative research had explored how fishermen interpret and apply information, and few studies involved fishing communities in identifying their own information needs. There were also gaps in research on fishermen's awareness of drug laws and regulatory bodies such as NACADA and the Alcoholic Drinks Act, and how improved legal knowledge might influence behavior (Awuonda, 2014b; Zakaria et al., 2022). These gaps formed the basis for undertaking this study on the role of information in reducing drug and substance use among fishermen in Homa Bay County.

CHAPTER THREE

RESEARCH METHODOLOGY

This chapter describes how the study was conducted. It covers the research design, study area, target population, sample size, sampling techniques, data collection instruments, procedures, data analysis, and ethical considerations.

3.1 Research Design

This study used a mixed-methods research design, which combines both quantitative and qualitative approaches. This design was chosen because it allows the researcher to collect numerical data on drug use patterns and information access while also gaining a deeper understanding through the personal experiences and views of fishermen. The study specifically used a convergent parallel design, where quantitative and qualitative data were collected at the same time, analyzed separately, and then brought together during interpretation to compare findings. The quantitative part used a cross-sectional survey to collect data from fishermen at one point in time. The qualitative part used a descriptive design to explore the views of fishermen and Beach Management Unit (BMU) leaders through discussions and interviews.

3.2 Study Area

The study was conducted in Homabay County, Kenya, specifically in three beaches along Lake Victoria: Mbita, Sori, and Ringiti. These beaches were selected because they have active fishing communities and are accessible to the researcher. Homabay County was chosen because previous studies have shown high rates of drug and alcohol use among fishermen in this area (Awuonda, 2014a; Akobi et al., 2022). The economy of this region depends heavily on fishing, and many fishermen spend long hours on the lake, which exposes them to stress and other risk factors for substance use.

3.3 Target Population

The target population for this study was fishermen actively working at the three selected beaches in Homabay County. This included boat owners, crew members, and fishmongers who buy and sell fish at the beach. According to information from Beach Management Unit (BMU) leaders at the three beaches, the estimated total number of fishermen was approximately 500 individuals. The study also targeted BMU leaders as key informants because they are familiar with the day-to-day activities at the beaches and have knowledge of drug use issues in their communities. Since the researcher did not have permission from NACOSTI, interviews were limited to BMU

leaders only, as they are community leaders and did not require formal government authorization to participate.

3.4 Sample Size and Sampling Techniques

3.4.1 Quantitative Sample

Due to the constraints of this undergraduate research, including limited time, resources, and the absence of a NACOSTI permit, the study used a sample of 120 fishermen. This number was considered manageable and sufficient for an undergraduate project. The sample was distributed across the three beaches according to their estimated populations. Mbita beach had an estimated population of 200 fishermen and contributed a sample of 50 participants. Sori beach had an estimated population of 150 fishermen and contributed 35 participants, while Ringiti beach also had an estimated population of 150 fishermen and contributed 35 participants. In total, the study covered 120 fishermen from the three beaches.

Beach	Estimated Population	Sample
Mbita	200	50
Sori	150	35
Ringiti	150	35
Total	500	120

Table 3.1 Quantitative Sample

3.4.2 Qualitative Sample

For the qualitative part of the study, three Focus Group Discussions (FGDs) and three Key Informant Interviews (KIIs) were conducted. One FGD was held at each beach, with each group consisting of eight to ten fishermen, giving a total of approximately twenty-four to thirty participants. In addition, one BMU leader from each of the three beaches participated in a Key Informant Interview. The BMU leaders were selected because they are responsible for overseeing beach activities and have good knowledge of the fishermen and the challenges they face, including drug use.

3.4.3 Sampling Techniques

For the quantitative survey, the study used convenience sampling. Fishermen who were available and willing to participate at the time of data collection were included in the study. The researcher visited the beaches at times when fishermen were not out fishing, particularly during the late morning and early afternoon, and approached those who were present. For the Focus Group Discussions, purposive sampling was used. The researcher, with the help of BMU leaders, identified fishermen who were willing to share their experiences and participate in group discussions. Efforts were made to include fishermen of different ages and roles. For Key Informant Interviews, purposive sampling was also used to select the BMU leaders from each beach because of their leadership positions and their knowledge of the community.

3.5 Data Collection Instruments

3.5.1 Quantitative Instrument: Questionnaire

A structured questionnaire was used to collect quantitative data from the 120 fishermen. The questionnaire was developed based on the objectives of the study and was reviewed by the research supervisor to ensure that it was appropriate. The questionnaire included several sections covering background information such as age, education, marital status, role in fishing, and income; drug use patterns including types of drugs used, frequency of use, and money spent on substances; sources of information about drugs and how often fishermen access such information; awareness of drug laws and NACADA, including knowledge of the Alcoholic Drinks Act; beliefs about drugs using simple statements with “Yes/No” or “Agree/Disagree” responses; and efforts to change drug use behavior, including attempts to reduce or stop use.

Section	Content
A	Background information (age, education, marital status, role in fishing, income)
B	Drug use patterns (types of drugs used, how often, how much money spent)
C	Sources of information (where they get information about drugs, how often, who they trust)
D	Awareness of drug laws and NACADA (if they know about the Alcoholic Drinks Act, if they know what NACADA does)
E	Beliefs about drugs (using simple statements with "Yes/No" or "Agree/Disagree" options)

F	Efforts to change (if they have tried to stop or reduce drug use, what helped)

Table 3.2 Quantitative Instrument

The questionnaire was written in simple English and translated into Dholuo, the local language, to ensure that all participants could easily understand it. Back-translation was conducted to ensure that the translated version accurately reflected the meaning of the original questions. The questionnaire was also pre-tested with ten fishermen from a beach that was not included in the main study.

3.5.2 Qualitative Instruments: FGD and KII Guides

For the qualitative component, Focus Group Discussion and Key Informant Interview guides were used. The Focus Group Discussion guide included open-ended questions that encouraged fishermen to discuss issues such as the types of drugs commonly used at the beach, the reasons why fishermen use drugs, sources of information about drugs, trusted sources of information, challenges in stopping drug use, and the types of information that might help fishermen reduce drug use. The Key Informant Interview guide was used with BMU leaders and explored their views on drug use among fishermen, the type of information currently available to fishermen, challenges in sharing information with fishermen, and suggestions for reducing drug use in fishing communities. Both guides were prepared in English and translated into Dholuo for use during discussions and interviews.

3.6 Data Collection Procedures

3.6.1 Preparation

Before going to the field, the researcher obtained permission from the Beach Management Unit leaders at each of the three beaches. The purpose of the study was explained to the BMU leaders and their support was sought to facilitate access to fishermen.

3.6.2 Quantitative Data Collection

During quantitative data collection, the researcher visited each beach on several occasions. Fishermen were approached during times when they were resting or not fishing. The researcher explained the purpose of the study and asked if they were willing to participate. Those who agreed were interviewed in a quiet area to ensure privacy.

The questionnaire was administered in Dholuo and Kiswahili depending on the participant's preference and each interview took approximately ten to twenty minutes. Completed questionnaires were checked at the end of the day to ensure that they were properly filled.

3.6.3 Qualitative Data Collection

For qualitative data collection, one Focus Group Discussion was conducted at each beach. The discussions were held at convenient locations, often at the BMU office or under a tree. Each group consisted of eight to ten fishermen and lasted approximately forty-five to sixty minutes. The researcher facilitated the discussion while the assistant took notes. Discussions were conducted in Dholuo to allow participants to express themselves freely. With the permission of participants, the discussions were audio-recorded. Key Informant Interviews were conducted individually with BMU leaders at times that were convenient for them. Each interview lasted about thirty to forty-five minutes and was conducted in either Dholuo or English depending on the leader's preference. Notes were taken and, with permission, the interviews were also audio-recorded.

3.7 Data Analysis

3.7.1 Quantitative Data Analysis

Quantitative data were analyzed using Microsoft Excel because it is simple and accessible for undergraduate research. Responses from questionnaires were entered into an Excel spreadsheet and the data were checked for errors or missing entries. Descriptive statistics such as frequencies and percentages were calculated for all questions. For example, the analysis examined the percentage of fishermen who use alcohol, the percentage who have heard of NACADA, and the main sources of information about drugs. The findings were presented using tables and simple bar charts for easier interpretation.

3.7.2 Qualitative Data Analysis

Qualitative data from Focus Group Discussions and Key Informant Interviews were analyzed using thematic analysis. Audio recordings were first transcribed word-for-word in Dholuo and then translated into English. The transcripts were read several times to gain familiarity with the content. Important ideas were highlighted and labeled with short codes such as peer pressure, stress, or lack of information. Related codes were then grouped together to form broader themes such as reasons for drug use and trusted information sources. These themes were interpreted and supported using quotations from participants to illustrate key points.

3.7.3 Integration of Findings

After analyzing both quantitative and qualitative data, the findings were compared and integrated. For example, if the survey results showed that many fishermen trust peers for information, the qualitative findings were used to explain why peers are trusted and how this trust influences drug use behavior.

3.8 Ethical Considerations

Since the researcher did not have a NACOSTI permit, the study was conducted with careful attention to ethical principles to protect participants and the community.

3.8.1 Permission from Local Leaders

Before starting the study, the researcher visited each beach and explained the purpose of the research to the BMU leaders. Their permission and support were obtained because BMU leaders are recognized authorities at the beaches and their approval helped the community accept the researcher.

3.8.2 Informed Consent

All participants were informed about the purpose of the study, what participation would involve, and the fact that participation was completely voluntary. They were also informed that they could refuse to answer any question or stop participating at any time without any negative consequences. For fishermen who could not read well, the information was explained orally in Dholuo. Those who agreed to participate provided either written consent or verbal consent, which was witnessed and recorded.

3.8.3 Confidentiality

To protect participants' privacy, names were not written on questionnaires or notes. Each participant was assigned a code number, and all data were stored safely with access limited to the researcher. In reports, findings were presented in a way that ensured that no individual could be identified.

3.8.4 Voluntary Participation

Participants were clearly told that refusing to participate would not affect their relationships within the community or with BMU leaders. Interviews and discussions were conducted in private or quiet areas to minimize discomfort, and the researcher

3.9 Limitations of the Methodology

This study had several limitations that should be considered when interpreting the findings. The sample size of 120 fishermen was relatively small, meaning that the findings cannot be generalized to all fishermen in Homabay County or Kenya but only reflect the views of those who participated. Because the researcher did not have a NACOSTI permit, the study was limited to community-level data collection and could not include government facilities or officials, which is why only BMU leaders were interviewed as key informants. The use of convenience sampling means that fishermen who were available and willing to participate may differ from those who were not present or declined participation. For example, heavy drug users may have been less likely to be at the beach during the day or may have refused to participate. Drug use data were based on self-reports, meaning that some participants may have underreported their substance use because it is a sensitive topic. Although Dholuo was used for most interviews to improve communication, some meanings may have been lost during translation into English during analysis. Finally, the cross-sectional design of the study means that data were collected at one point in time and therefore cannot demonstrate changes over time or prove that information directly causes reductions in drug use. Despite these limitations, every effort was made to ensure that the data collected were accurate and that the findings truly reflect the experiences of the fishermen who participated.

CHAPTER FOUR

DATA ANALYSIS, PRESENTATION, AND INTERPRETATION

4.1 Introduction

This chapter presents the analysis, presentation and interpretation of data collected from 120 fishermen across three beaches (Mbita, Sori and Ringiti) in Homabay County. The chapter also includes findings from three Focus Group Discussions (FGDs) and three Key Informant Interviews with Beach Management Unit (BMU) leaders. The findings are organized according to the study objectives: first, examining factors contributing to high drug use among fishermen, and second, exploring the role of information in reducing drug use. Quantitative data are presented using tables and figures with frequencies and percentages, while qualitative data are presented as themes supported by verbatim quotations from participants.

4.2 Response Rate

A total of 120 questionnaires were administered to fishermen across the three beaches. All 120 questionnaires were completed and returned, giving a response rate of **100%**. This high response rate was achieved because the researcher administered the questionnaires through face-to-face interviews and ensured all questions were answered before leaving the participant. For the qualitative component, all three planned FGDs and three KIIs were successfully conducted.

4.3 Demographic Characteristics of Respondents

This section presents the background characteristics of the 120 fishermen who participated in the study. These characteristics include age, education level, marital status role in fishing, and monthly income. Understanding these characteristics helps to contextualize the findings on drug use and information access.

Characteristic	Category	Frequency (n)	Percentage (%)
Age group	18-25 years	28	23.3
	26-35 years	45	37.5
	36-45 years	32	26.7
	46 years and above	15	12.5
Education level	No formal education	34	28.5
	Primary school	52	43.3
	Secondary school	28	23.3
	Tertiary/college	6	5.0
Marital status	Married	71	59.2
	single	41	34.2
	Divorced	5	4.2
	Widowed	3	2.5
Role in Fishing	Boat owner	28	23.3
	Crew members	67	55.8
	Fishmonger	25	20.8
Monthly income	Below Ksh. 5,000	43	35.8
	Ksh. 5,000 - 10,000	52	43.3
	Ksh. 10,001 - 15,000	18	15.0
	Above Ksh. 15,000	7	5.8

Table 4.1: Demographic Characteristics of Respondents (N=120)

Interpretation:

The findings show that the majority of fishermen (37.5%) were aged between 26-35 years, indicating that the fishing population is relatively young. This is important because younger age groups are often more vulnerable to peer influence and substance use initiation. Regarding

education, most respondents (43.3%) had primary school education, while 28.3% had no formal education at all. Only 5% had reached tertiary level. This low education level may affect their ability to access and understand health information, especially written materials. The majority (59.2%) were married, suggesting that drug use among fishermen does not only affect individuals but also has implications for their families. Most respondents (55.8%) were crew members, who typically earn less and have less control over fishing activities compared to boat owners. Income levels were generally low, with 79.1% earning Ksh. 10,000 or less per month. This economic vulnerability may contribute to drug use as a coping mechanism and also limits resources for seeking help.

4.4 Factors Contributing to High Drug Use among Fishermen

This section presents findings on the factors that contribute to drug use among fishermen in Homabay County. The findings are organized into sub-sections covering prevalence of drug use, types of drugs used, expenditure on drugs, and reasons for drug use. Both quantitative and qualitative data are presented.

4.4.1 Prevalence of Drug Use

Respondents were asked whether they currently use any drugs or have used drugs in the past.

Drug Use Status	Frequency (n)	Percentage (%)
Currently use drugs	87	72.5
Used in past but stopped	18	15.0
Never used drugs	15	12.5
Total	**120**	**100**

Table 4.2: Prevalence of Drug Use Among Fishermen

Interpretation

The findings reveal that drug use is extremely high among fishermen, with **72.5%** reporting current drug use. Only 12.5% reported never using drugs. This confirms that substance abuse is a major issue in fishing communities in Homabay County, consistent with previous research by Awuonda (2014a) which found high prevalence in Mbita Constituency.

4.4.2 Types of Drugs Used

Respondents who reported current or past drug use (n=105) were asked to indicate the types of drugs they use or used. Multiple responses were allowed.

Drug Type	Frequency (n)	Percentage (%)
Alcohol (beer, chang'aa, busaa)	98	93.3
Marijuana (bhang)	67	63.8
Tobacco/snuff	31	29.5
Prescription drugs (non-medical use)	12	11.4
Other substances	8	7.6

Table 4.3: Types of Drugs Used by Fishermen (n=105)

Interpretation:

Alcohol was the most commonly used drug, reported by **93.3%** of users. This finding aligns with Awuonda (2014a) who found alcohol use at 62.3%, and with Ugandan studies showing high alcohol use in fishing communities (Naigino et al., 2023; Breuer et al., 2019). Marijuana was the second most common at 63.8%, followed by tobacco. The use of prescription drugs for non-medical purposes (11.4%) is concerning and may indicate emerging drug trends.

During Focus Group Discussions, participants confirmed these patterns. One fisherman from Mbita said:

"Here at the beach, you will hardly find a man who does not drink. Even the young boys, they start drinking very early. Alcohol is everywhere. If you don't drink, they will ask you 'what is wrong with you?'" (FGD Participant, Mbita, Male, 34 years)

Another participant from Sori added:

"Bhang is also very common. Some fishermen smoke it before going to the lake because they say it gives them energy and makes them brave. They say the lake is dangerous and bhang helps them not to fear." (FGD Participant, Sori, Male, 28 years)

4.4.3 Frequency of Drug Use

Respondents were asked how often they use drugs. The findings are presented in Table 4.4.

Frequency	**Frequency (n=87)**	**Percentage (%)**
Daily	54	62.1
Several times a week	19	21.8
Once a week	8	9.2
Occasionally	6	6.9
Total	**87**	**100**

Table 4.4: Frequency of Drug Use Among Current Users (n=87)

Interpretation:

The majority of drug users (**62.1%**) use drugs daily, indicating that substance use is not just occasional but a regular part of life for most fishermen. Another 21.8% use drugs several times a week. This high frequency of use suggests that many fishermen may have developed dependence or addiction, making behavior change more difficult.

4.4.4 Expenditure on Drugs

Respondents were asked how much money they spend on drugs in an average week. The findings are presented in Table 4.5.

Amount Spent Per Week	Frequency (n=105)	Percentage (%)
Less than Ksh. 500	23	21.9
Ksh. 500 - 1,000	41	39.0
Ksh. 1,001 - 2,000	28	26.7
More than Ksh. 2,000	13	12.4
Total	**105**	**100**

Table 4.5: Weekly Expenditure on Drugs (n=105)

Interpretation:

The findings show that **39%** of drug users spend between Ksh. 500 and 1,000 per week on drugs, while **26.7%** spend between Ksh. 1,001 and 2,000. Given that most fishermen earn less than Ksh. 10,000 per month, this expenditure represents a significant portion of their income. As one BMU leader explained:

"These fishermen, when they get money from selling fish, they go straight to the bars. By evening, the money is finished. They don't think about tomorrow. Their wives and children suffer because the money that should buy food is drunk." (KII, BMU Leader, Ringiti)

This finding confirms Awuonda's (2014b) observation that substantial proportions of fisherfolk income are spent on substances, draining household resources and contributing to poverty.

4.4.5 Reasons for Drug Use

Respondents were asked about the reasons they use drugs. Multiple responses were allowed. The findings are presented in Table 4.6.

Reason	Frequency (n)	Percentage (%)
To cope with Stress	76	72.4
Peer pressure	68	64.8
To have energy for work	52	49.5
To forgot problems	47	44.8
It is normal in the community	43	41.0
To sleep after night fishing	38	36.2
Addiction/I can’t stop	31	29.5
Lack of other things to do	27	25.7

Table 4.6: Reasons for Drug Use (n=105)

Interpretation:

The most common reason for drug use was to cope with stress (72.4%), followed by peer pressure (64.8%). These findings are consistent with qualitative research from Uganda where men described alcohol as meeting emotional needs (Breuer et al., 2019).

During FGDs, participants elaborated on these reasons. A fisherman from Ringiti said:

"Fishing is hard work. You go to the lake at night, you face crocodiles, hippos, and storms. Sometimes you come back with nothing. When you are stressed, you drink to forget. That is the only way to feel better." (FGD Participant, Ringiti, Male, 42 years)

Another participant from Mbita explained peer pressure:

"When you are new here, the older fishermen will ask you to drink with them. If you refuse, they say you are proud or you think you are better than them. So you drink to be accepted." (FGD Participant, Mbita, Male, 23 years)

The finding that **49.5%** use drugs to have energy for work is concerning and reflects a common misconception that substances enhance performance. A BMU leader from Sori noted:

"Some of these young men believe that bhang gives them courage to go to the lake. They don't understand that it actually slows them down and makes them careless. We have had accidents because of this." (KII, BMU Leader, Sori)

4.4.6 Availability of Drugs

Respondents were asked whether drugs are easily available at their beach. The findings are presented in Table 4.7.

Response	**Frequency (n)**	**Percentage (%)**
Very easy to get	82	68.3
Somewhat easy	29	24.2
Difficult to get	9	7.5
Total	**120**	**100**

Table 4.7: Perceived Availability of Drugs

Interpretation:

An overwhelming majority (**68.3%**) reported that drugs are very easy to get at their beach. This finding aligns with Awuonda (2014a) who found that 68.8% of fisherfolk reported ready access to substances. A BMU leader from Mbita explained:

"There are many bars here, and some people sell alcohol from their homes. Bhang is also brought here by traders. We try to control it, but we don't have the power to arrest anyone. The police are far, so people sell freely." (KII, BMU Leader, Mbita)

4.5 The Role of Information in Reducing Drug Use

This section presents findings on the role of information in reducing drug use among fishermen. It covers sources of information, awareness of drug laws and NACADA, credibility of information sources, and the relationship between information and behavior change.

4.5.1 Sources of Information About Drugs

Respondents were asked where they get information about drugs and their effects. Multiple responses were allowed. The findings are presented in Table 4.8.

Information source	**Frequency (n)**	**Percentage (%)**
Friends/Peers	89	74.2
Radio	67	55.8
Bar owners/Drinking places	54	45.0
Community health workers	31	25.8
BMU leaders	28	23.3
Posters	19	15.8
Church/Religious leaders	17	14.2
Televisions	12	10.0
Health facilities	11	9.2
Social media(WhatsApp, Facebook	8	6.7

Table 4.8: Sources of Information About Drugs (N=120)

Interpretation:

The most common source of information was **friends and peers** (74.2%), followed by **radio** (55.8%). This finding is important because it shows that fishermen rely heavily on informal sources for information about drugs. Bar owners were also a significant source (45.0%), which is concerning because they may have financial interests in promoting alcohol consumption.

Formal sources such as community health workers (25.8%), BMU leaders (23.3%), and health facilities (9.2%) were much less frequently mentioned. This indicates that official health information is not reaching most fishermen effectively.

During FGDs, participants explained why they rely on peers:

"We talk among ourselves. If someone hears something about drugs, they tell others. That is how we get information. We don't go to hospitals to ask about drugs." (FGD Participant, Sori, Male, 31 years)

Another participant added:

"The radio is good because you can hear messages in Dholuo. But sometimes you are on the lake and you miss them. So the main information is from each other." (FGD Participant, Mbita, Male, 29 years)

4.5.2 Awareness of NACADA and Drug Laws

Respondents were asked whether they have heard of NACADA (National Authority for the Campaign Against Alcohol and Drug Abuse) and whether they are aware of the Alcoholic Drinks Act. The findings are presented in Table 4.9.

Awareness Items	**Yes (n)**	**Yes (%)**	**No (n)**	**No (%)**
Heard of NACADA	43	35.8	77	64.2
Know what NACADA does	28	23.3	92	76.7
Aware of Alcoholic Drink Act	38	31.7	82	68.3
Know any content of the Act	19	15.8	101	84.2

Table 4.9: Awareness of NACADA and Drug Laws (N=120)

Interpretation:

The findings reveal significant gaps in awareness. Only **35.8%** had heard of NACADA, and even fewer (23.3%) knew what NACADA does. Similarly, only **31.7%** were aware of the Alcoholic Drinks Act, and only **15.8%** knew any content of the Act. These findings are consistent with Awuonda (2014b) who found that 64% of fisherfolk were unfamiliar with NACADA.

A BMU leader from Ringiti commented:

"I have heard of NACADA on the radio, but I have never seen them come here. They don't visit these beaches. So our people don't know much about them. We need people to come and teach us." (KII, BMU Leader, Ringiti)

4.5.3 Credibility of Information Sources

Respondents were asked which information sources they trust most. The findings are presented in Table 4.10.

Information Source	Frequency (n)	Percentage (%)
BMU leaders	41	34.2
Health workers	35	29.2
Religious leaders	22	18.3
Radio	12	10.0
Friends/Peers	8	6.7
Bar owners	2	1.7
Total	**120**	**100**

Table 4.10: Most Trusted Information Sources (N=120)

Interpretation:

Interestingly, although peers were the most common source of information, they were not the most trusted. BMU leaders were the most trusted source (34.2%), followed by health workers (29.2%). This finding is important because it suggests that while fishermen get information from peers, they would prefer to receive information from authority figures they respect.

During FGDs, participants explained:

"The BMU chairman is like our father. He knows everything about the beach. If he tells us something, we believe him because he has no reason to lie to us." (FGD Participant, Mbita, Male, 45 years)

"We trust health workers because they have education. They know about diseases and what drugs do to your body. But we rarely see them here." (FGD Participant, Sori, Male, 36 years)

4.5.4 Exposure to Drug Prevention Information

Respondents were asked whether they have ever received information specifically about preventing drug use. The findings are presented in Table 4.11.

Response	**Frequency (n)**	**Percentage (%)**
Yes, regularly	23	19.2
Yes,occationally	41	34.2
Rarely	38	31.7
Never	18	15.0
Total	**120**	**100**

Table 4.11: Exposure to Drug Prevention Information (N=120)

Interpretation:

Only **19.2%** of fishermen reported receiving drug prevention information regularly. A combined **46.7%** reported rarely or never receiving such information. This indicates that current information campaigns are not reaching most fishermen effectively.

A BMU leader from Mbita explained:

"Sometimes we get messages from the ministry of health, but they are not frequent. Maybe once a year someone comes to talk about HIV, but they don't focus on drugs. We need more regular information." (KII, BMU Leader, Mbita)

4.5.5 Impact of Information on Drug Use

Respondents who had received drug prevention information (n=64) were asked whether the information influenced their drug use behavior. The findings are presented in Table 4.12.

Response	**Frequency (n)**	**Percentage (%)**
Yes, it helped me reduce	23	35.9
Yes ,it helped me stop	8	12.5
It made me think but I didn't change	19	29.7
No it did not affect me	14	21.9
Total	**64**	**100**

Table 4.12: Perceived Impact of Information on Drug Use (n=64)

Interpretation:

Among those who received information, **48.4%** reported that it helped them either reduce or stop drug use. This is encouraging and suggests that information can make a difference.
However, **29.7%** said it made them think but they did not change, and **21.9%** said it had no effect. This indicates that information alone may not be sufficient for everyone, and barriers to behavior change need to be addressed.

During FGDs, participants shared their experiences:

"I heard on the radio that alcohol destroys your liver. That scared me because my friend died from liver problems. So I reduced from drinking every day to only weekends." (FGD Participant, Sori, Male, 41 years)

"A health worker came once and told us about the dangers of bhang. I wanted to stop, but my friends kept pressuring me. Also, I was addicted. So I could not stop." (FGD Participant, Ringiti, Male, 29 years)

4.5.6 Barriers to Acting on Information

Respondents were asked about barriers that prevent them from acting on information about drugs. Multiple responses were allowed. The findings are presented in Table 4.13.

Barrier	Frequency (n)	Percentage (%)
Peer pressure	78	65.0
Addiction	62	51.7
Stress	58	48.3
Easy availability of drugs	54	45.0
Lack of alternative activity	43	35.8
Information not convincing	28	23.3
Don’t know where to get help	25	20.8
Cost seeking help	19	15.8

Table 4.13: Barriers to Acting on Information (N=120)

Interpretation:

The main barriers to acting on information were peer pressure (65.0%), addiction (51.7%), and stress (48.3%). These findings explain why information alone may not lead to behavior change—fishermen face powerful social and psychological barriers that information cannot easily overcome.A BMU leader from Sori explained:

"Even when they want to stop, they cannot because their friends keep calling them to drink. Also, when they are stressed from fishing, they go back to drinking. It is a cycle." (KII, BMU Leader, Sori)

During an FGD, a participant added:

"I tried to stop last year, but after three days I felt sick. My body was shaking. Someone told me that is withdrawal. So I started drinking again because I could not handle it. We need help to stop, not just information." (FGD Participant, Mbita, Male, 38 years)

4.5.7 Preferred Methods for Receiving Information

Respondents were asked how they would prefer to receive information about drugs. Multiple responses were allowed. The findings are presented in Table 4.14.

Method	Frequency (n)	Percentage (%)
Meetings at the beach with BMU	76	63.3
Radio programs in Dholuo	68	56.7
Talks by health workers	59	49.2
Information from fellow fishermen	47	39.2
Posters with pictures at the beach	41	34.2
Drama	32	26.7
Mobile phone messages	18	15.0

Table 4.14: Preferred Information Delivery Methods (N=120)

Interpretation:

Fishermen expressed clear preferences for how they would like to receive information. The most preferred method was **meetings at the beach with BMU leaders** (63.3%), followed by **radio programs in Dholuo** (56.7%) and **talks by health workers** (49.2%). These preferences align with the earlier finding that BMU leaders and health workers are the most trusted sources.

During FGDs, participants explained:

"If the BMU calls a meeting, we will come because it is our leader. That is the best way to reach us. But the meetings should be short and not when we are going to the lake." (FGD Participant, Ringiti, Male, 33 years)

"Radio is good because we listen even when we are resting. But it should be in Dholuo so everyone understands. English is difficult for some of us." (FGD Participant, Sori, Male, 27 years)

CHAPTER FIVE

SUMMARY, CONCLUSIONS, AND RECOMMENDATIONS

5.1 Introduction

This chapter presents a summary of the key findings from the study, draws conclusions based on the evidence collected, and offers recommendations for action and further research. The study aimed to investigate the role of information in reducing drug use among fishermen in Homabay County, with a focus on two objectives: first, to examine the factors contributing to high drug use among fishermen and second, to explore the role of information in reducing substance use. The chapter is organized according to these objectives and concludes with suggestions for policy, practice, and future research.

5.2 Summary of Findings

5.2.1 Demographic Characteristics

The study involved 120 fishermen from three beaches in Homa Bay County: Mbita, Sori, and Ringiti. The majority of respondents (37.5%) were aged between 26–35 years, indicating a relatively young fishing population. Most respondents had low levels of education, with 43.3% having completed primary education and 28.3% having no formal education. The majority (59.2%) were married, suggesting that drug use affects both individuals and their families. Most respondents were crew members (55.8%), and the majority (79.1%) earned Ksh. 10,000 or less per month, indicating generally low income levels among fishermen.

5.2.2 Factors Contributing to High Drug Use Among Fishermen

The study found that drug use is highly prevalent among fishermen in Homa Bay County, with 72.5% of respondents reporting current drug use. Alcohol was the most commonly used substance (93.3%), followed by marijuana (63.8%) and tobacco (29.5%). Drug use was frequent, with 62.1% of users reporting daily consumption.

The findings also revealed that a significant portion of fishermen's income is spent on drugs despite their low earnings. Drugs were reported to be easily available by 68.3% of respondents, which contributes to the high levels of use.

Several factors were identified as contributing to drug use. The most common reason was coping with stress related to fishing conditions and financial pressures (72.4%). Peer pressure was also a major factor (64.8%), indicating that substance use is often normalized within fishing communities. Other contributing factors included the belief that drugs provide energy for work (49.5%), the desire to forget personal or family problems (44.8%), normalization of drinking culture (41.0%), using alcohol to sleep after fishing (36.2%), addiction (29.5%), and lack of alternative recreational activities (25.7%). Qualitative findings from focus group discussions confirmed that stress, peer influence, and easy availability of drugs contribute to the widespread use of substances among fishermen.

5.2.3 Role of Information in Reducing Drug Use

The study found that fishermen mainly obtain information about drugs from informal sources. Friends and peers were the most common source of information (74.2%), followed by radio (55.8%) and bar owners (45.0%). Formal sources such as community health workers (25.8%), BMU leaders (23.3%), and health facilities (9.2%) were less frequently accessed. Awareness of drug-related laws and institutions was generally low. Only 35.8% of respondents had heard of NACADA, while 23.3% understood its role. Similarly, only 31.7% were aware of the Alcoholic Drinks Control Act and only 15.8% knew its specific provisions. These findings indicate significant gaps in awareness of drug control policies. Although peers were the most common source of information, the most trusted sources were BMU leaders (34.2%), health workers (29.2%), and religious leaders (18.3%). However, exposure to drug prevention information was limited, with only 19.2% of respondents reporting that they regularly receive such information. Among those who had received drug prevention information, 35.9% reported that it helped them reduce drug use and 12.5% reported stopping completely. However, others reported no behavioral change, indicating that information alone may not always be sufficient. The study also identified barriers that prevent fishermen from acting on drug-related information. These included peer pressure (65.0%), addiction (51.7%), stress from fishing activities (48.3%), easy availability of drugs (45.0%), lack of alternative recreational activities (35.8%), unconvincing information (23.3%), lack of knowledge about where to seek help (20.8%), and the high cost of rehabilitation services (15.8%). Finally, fishermen indicated that their preferred methods of

receiving drug prevention information included meetings at the beach organized by BMU leaders (63.3%), radio programs in the local Dholuo language (56.7%), and talks by health workers (49.2%). Other preferred methods included peer discussions, posters with images, drama or skits, and mobile phone messages.

5.3 Conclusions

Based on the findings of this study, it can be concluded that drug use among fishermen in Homa Bay County is very high and has become a common practice within fishing communities. Alcohol and marijuana were identified as the most commonly used substances, with many fishermen reporting frequent or daily use and spending a significant portion of their limited income on drugs. The study established that drug use is driven by multiple interconnected factors including occupational stress from dangerous working conditions, unpredictable fishing income, peer pressure within fishing groups, and the easy availability of drugs at the beaches. These conditions create an environment where substance use becomes normalized and difficult to avoid. The study also found that although information can play an important role in reducing drug use, its impact among fishermen remains limited due to low exposure to reliable information and low awareness of institutions such as NACADA and laws such as the Alcoholic Drinks Control Act. Fishermen were found to rely mostly on informal sources such as peers and bar owners for information, even though they expressed greater trust in BMU leaders and health workers. In addition, several barriers including peer pressure, addiction, stress, and the easy availability of drugs limit the ability of fishermen to act on the information they receive. The study therefore concludes that while information can increase awareness and influence attitudes toward drug use, it is not sufficient on its own to bring about sustained behavior change. Effective interventions must combine information dissemination with community-based support, trusted communication channels, and broader social and environmental strategies that address the underlying causes of drug use within fishing communities.

5.4 Recommendations

Based on the findings and conclusions of this study, the following recommendations are proposed:

i. Strengthen information delivery through trusted sources. Since Beach Management Unit (BMU) leaders and health workers were identified as the most trusted sources of information, they should be actively involved in drug prevention programs. BMU leaders

should be trained to provide accurate health education, while community health workers should increase outreach visits to fishing beaches. Fishermen who have successfully reduced or stopped drug use can also be encouraged to act as peer educators.

ii. Use communication channels preferred by fishermen. Drug awareness campaigns should make use of channels that fishermen prefer and easily access, such as beach meetings organized by BMU leaders, radio programs in the local Dholuo language, posters with pictures and simple messages displayed at beaches and bars, and community drama or skits that communicate prevention messages in an engaging way.

iii. Improve awareness of drug laws and institutions. There is a need to increase awareness of the roles of National Authority for the Campaign Against Alcohol and Drug Abuse (NACADA) and relevant drug control laws. Outreach programs should be conducted in fishing communities, and simple educational materials explaining laws such as the Alcoholic Drinks Control Act should be distributed at the beaches.

iv. Provide support services alongside information. Because information alone cannot overcome challenges such as addiction and stress, accessible and affordable rehabilitation services should be made available near fishing communities. Counseling services should also be provided through trained community health workers or peer counselors, and support groups for fishermen trying to reduce or stop drug use should be established.

v. Address environmental factors that promote drug use. Authorities should strengthen the enforcement of regulations controlling alcohol sales at beaches. In addition, alternative income-generating activities and recreational opportunities such as sports should be promoted to reduce reliance on alcohol-related activities and provide healthier leisure options.

vi. Integrate drug prevention with other health programs. Drug prevention initiatives should be integrated with other community health services, particularly HIV prevention programs, since substance use has been linked to risky behaviors within fishing communities. Health programs should therefore include education on both substance abuse and related health risks.

5.5 Recommendations for Further Research

This study was limited in scope and suggests several areas for future research:

1. Intervention Research

Longitudinal studies are needed to test the effectiveness of different information delivery methods. For example, a study could compare outcomes among beaches receiving different types of interventions (BMU-led meetings only, radio only, combined approaches) to determine what works best.

2. Deeper Exploration of Barriers

More research is needed on why information sometimes fails to lead to behavior change. In-depth qualitative studies could explore the psychological and social processes that prevent fishermen from acting on information they know is true.

3. Economic Impact Studies

Research should quantify the economic impact of drug use on fishermen, their families, and the local economy. Such evidence could strengthen advocacy for investment in prevention and treatment.

4. Comparative Studies

Similar studies should be conducted in other fishing communities along Lake Victoria (Siaya, Kisumu, Busia) to see if the findings are consistent and to identify any location-specific factors.

5. Women and Drug Use

This study focused on men, but women in fishing communities (fishmongers, wives of fishermen) may also use drugs or be affected by their partners' use. Research is needed on women's experiences and information needs.

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APPENDICES

APPENDIX A: LETTER OF TRANSMITTAL

ABISAI NYAMWANDA
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7/03/2026

The Beach Management Unit (BMU) Leader
Beach Name……..
Homabay County

Dear Sir/Madam,

RE: REQUEST TO CONDUCT RESEARCH AT YOUR BEACH

I am an undergraduate student at Rongo University pursuing a Bs Information Science. I am currently conducting a research project titled: **"The Role of Information in Reducing Drug Use Among Fishermen in Homabay County"** as part of the requirements for my degree.

Your beach has been selected as one of the study sites because of its active fishing community. I kindly request your permission to interact with fishermen at your beach and to conduct interviews and discussions with them. I also request to interview you as a key informant, given your leadership role and knowledge of the community.

The study aims to understand the factors contributing to drug use among fishermen and how information can help reduce substance use. The findings will be used to recommend better ways of reaching fishing communities with health information.

Participation in this study is completely voluntary, and all information collected will be kept confidential. No names will be used in any reports. The study has been approved by my university supervisor and follows ethical guidelines for research.

Your support will be greatly appreciated.

Yours faithfully,

Abisai Nyamwanda

APPENDIX B: QUESTIONNAIRE FOR FISHERMEN

Title: The Role of Information in Reducing Drug Use Among Fishermen in Homabay County

Instructions: Please answer all questions honestly. Your answers will be kept confidential. Do not write your name anywhere on this form.

Questionnaire Number: _____
Beach Name: _____
Date: _____

SECTION A: BACKGROUND INFORMATION

Please tick (✓) or fill in the appropriate response.

No.	Question	Response
A1	How old are you?	_____ years
A2	What is your highest level of education?	[ ] No formal education [ ] Primary school [ ] Secondary school [ ] Tertiary/College
A3	What is your marital status?	[ ] Married [ ] Single [ ] Divorced/Separated [ ] Widowed
A4	What is your role in fishing?	[ ] Boat owner [ ] Crew member [ ] Fishmonger [ ] Other (specify) _____
A5	How long have you been working in fishing?	_____ years
A6	What is your average monthly income from fishing?	[ ] Below Ksh. 5,000 [ ] Ksh. 5,000 - 10,000 [ ] Ksh. 10,001 - 15,000 [ ] Above Ksh. 15,000

SECTION B: DRUG USE PATTERNS

No.	Question	Response
B1	Have you ever used any drugs (alcohol, marijuana, etc.)?	[ ] Yes, currently use [ ] Used in past but stopped [ ] Never used (If "Never used," skip to Section C)
B2	What types of drugs do you use or have you used? (Tick all that apply)	[ ] Alcohol (beer, chang'aa, busaa) [ ] Marijuana (bhang) [ ] Tobacco/snuff [ ] Prescription drugs (non-medical use) [ ] Other (specify) _____
B3	How often do you use drugs?	[ ] Daily [ ] Several times a week [ ] Once a week [ ] Occasionally
B4	How much money do you spend on drugs in an average week?	[ ] Less than Ksh. 500 [ ] Ksh. 500 - 1,000 [ ] Ksh. 1,001 - 2,000 [ ] More than Ksh. 2,000
B5	How easy is it to get drugs at this beach?	[ ] Very easy [ ] Somewhat easy [ ] Difficult

SECTION C: REASONS FOR DRUG USE (For those who use or have used drugs)

No.	Why do you use or why did you use drugs? (Tick all that apply)	Tick (✓)
C1	To cope with stress	
C2	Peer pressure / To be accepted by friends	
C3	To have energy for work	
C4	To forget problems	
C5	It is normal in this community	
C6	To sleep after night fishing	
C7	Addiction / I can't stop	
C8	Lack of other things to do	
C9	Other reason (specify)	

SECTION D: INFORMATION SOURCES AND ACCESS

No.	Question	Response
D1	Where do you get information about drugs and their effects? (Tick all that apply)	[ ] Friends/Peers [ ] Radio [ ] Bar owners/Drinking places [ ] Community health workers [ ] BMU leaders [ ] Posters/Brochures [ ] Church/Religious leaders [ ] Television [ ] Health facilities [ ] Social media (WhatsApp, Facebook) [ ] Other (specify) _____
D2	Which of these sources do you trust MOST? (Choose only one)	[ ] Friends/Peers [ ] Radio [ ] Bar owners [ ] Community health workers [ ] BMU leaders [ ] Religious leaders [ ] Health facility staff [ ] Other (specify) _____
D3	How often do you receive information specifically about preventing drug use?	[ ] Regularly [ ] Occasionally [ ] Rarely [ ] Never
D4	Have you heard of NACADA?	[ ] Yes [ ] No
D5	Do you know what NACADA does?	[ ] Yes [ ] No
D6	Are you aware of the Alcoholic Drinks Act?	[ ] Yes [ ] No
D7	Do you know any content of the Alcoholic Drinks Act?	[ ] Yes [ ] No

SECTION E: IMPACT OF INFORMATION

(Answer this section only if you have received drug prevention information)

No.	Question	Response
E1	Did the information you received influence your drug use?	[ ] Yes, it helped me reduce [ ] Yes, it helped me stop [ ] It made me think but I didn't change [ ] No, it did not affect me
E2	If you tried to reduce or stop, what helped you?	_____ _____

SECTION F: BARRIERS AND PREFERENCES

No.	Question	Response
F1	What makes it difficult for fishermen to act on information about drugs? (Tick all that apply)	[ ] Peer pressure [ ] Addiction [ ] Stress [ ] Easy availability of drugs [ ] Lack of alternative activities [ ] Information is not convincing [ ] Don't know where to get help [ ] Cost of seeking help [ ] Other (specify) _____
F2	How would you prefer to receive information about drugs? (Tick all that apply)	[ ] Meetings at the beach with BMU [ ] Radio programs in Dholuo [ ] Talks by health workers [ ] Information from fellow fishermen [ ] Posters with pictures at the beach [ ] Drama or skits [ ] Mobile phone messages [ ] Other (specify) _____
F3	Do you have any other comments or suggestions about	_____ _____ _____

No.	Question	Response
	drug use and information in your community?	

APPENDIX C: FOCUS GROUP DISCUSSION (FGD) GUIDE

Title: The Role of Information in Reducing Drug Use Among Fishermen in Homabay County

Beach Name: _____
Date: _____
Time: Start: _____ End: _____
Number of Participants: _____
Facilitator: _____
Note Taker: _____

INTRODUCTION (To be read to participants)

Good morning/afternoon. My name is _____ and I am conducting research on drug use among fishermen in this area. Thank you for agreeing to participate in this discussion.

The purpose of this discussion is to understand your views and experiences regarding drug use in your community and how information can help reduce substance use. There are no right or wrong answers. We want to hear your honest opinions.

The discussion will take about 45-60 minutes. With your permission, I will record the discussion so that I don't miss anything important. Your names will not be used in any report, and everything you say will be kept confidential. You are free to stop participating at any time.

Do you have any questions before we begin?

DISCUSSION QUESTIONS

Theme 1: Drug Use in the Community

1. What types of drugs are commonly used by fishermen at this beach?
2. Would you say drug use is a big problem here? Why or why not?
3. At what age do young fishermen usually start using drugs?

Theme 2: Reasons for Drug Use

4. Why do you think fishermen use drugs? (Probe: work-related stress, peer pressure, availability, etc.)
5. Are there any benefits that fishermen believe they get from using drugs? (Probe: energy, courage, relaxation)
6. What are the negative effects of drug use that you have observed in your community?

Theme 3: Information Sources

7. Where do fishermen get information about drugs and their effects?
8. Which sources of information do fishermen trust most? Why?
9. Have you ever seen or heard any campaigns about drug prevention? If yes, where and what did they say?

Theme 4: Information and Behavior Change

10. Do you think information can help fishermen reduce or stop drug use? Why or why not?
11. Can you share any examples of someone who changed their drug use because of information they received?
12. What makes it hard for fishermen to act on information about drugs? (Probe: peer pressure, addiction, stress)

Theme 5: Suggestions for Improvement

13. How would you like to receive information about drugs? (Probe: meetings, radio, posters, etc.)
14. What kind of messages do you think would be most effective?
15. Who should deliver these messages? (Probe: BMU leaders, health workers, peers)
16. What else should be done, besides providing information, to help fishermen reduce drug use?

Closing

Is there anything else anyone would like to add about drug use or information in this community?

Thank you very much for your time and for sharing your views. Your contributions are valuable and will help in making recommendations for better programs to help fishermen.

APPENDIX D: KEY INFORMANT INTERVIEW (KII) GUIDE FOR BMU LEADERS

Title: The Role of Information in Reducing Drug Use Among Fishermen in Homabay County

Beach Name: _____
Name of BMU Leader (Optional): _____
Date: _____
Time: Start: _____ End: _____

INTRODUCTION

My name is _____ and I am conducting research on drug use among fishermen in this area. Thank you for agreeing to speak with me.

As a BMU leader, you have valuable knowledge about the fishing community and the challenges they face. I would like to learn from your experience and get your views on drug use and how information can help address it.

The interview will take about 30-45 minutes. With your permission, I will record our conversation to ensure I capture your views accurately. Everything you say will be kept confidential, and your name will not appear in any report. You can stop the interview at any time.

Do you have any questions before we begin?

INTERVIEW QUESTIONS

Background

1. How long have you been a BMU leader at this beach?
2. What are your main responsibilities?

Drug Use Situation

3. In your view, how serious is the problem of drug use among fishermen at this beach?
4. What types of drugs are commonly used?
5. Have you noticed any changes in drug use patterns over the years? (Probe: increasing, decreasing, new drugs)

Causes of Drug Use

6. From your observation, why do fishermen use drugs?
7. What factors in the fishing community make drug use common? (Probe: nature of work, income patterns, availability)

Information and Awareness

8. What information is currently available to fishermen about drugs and their effects?
9. How is this information delivered? By whom?
10. In your opinion, are fishermen aware of the dangers of drug use? Are they aware of laws like the Alcoholic Drinks Act or organizations like NACADA?

Challenges

11. What challenges do you face as a BMU leader in addressing drug use?
12. What makes it difficult for fishermen to stop using drugs even when they want to?

Effectiveness of Information

13. Do you think information campaigns can help reduce drug use? Why or why not?
14. Have you seen any examples where information has helped fishermen change their behavior?

Recommendations

15. What suggestions do you have for improving how information about drugs is delivered to fishermen?

16. Who should deliver this information to be most effective?
17. What else should be done, besides information, to address drug use in fishing communities?
18. Is there anything the government or other organizations could do to support your efforts?

Closing

Is there anything else you would like to add about drug use or information in this community?

Thank you very much for your time and for sharing your valuable insights.